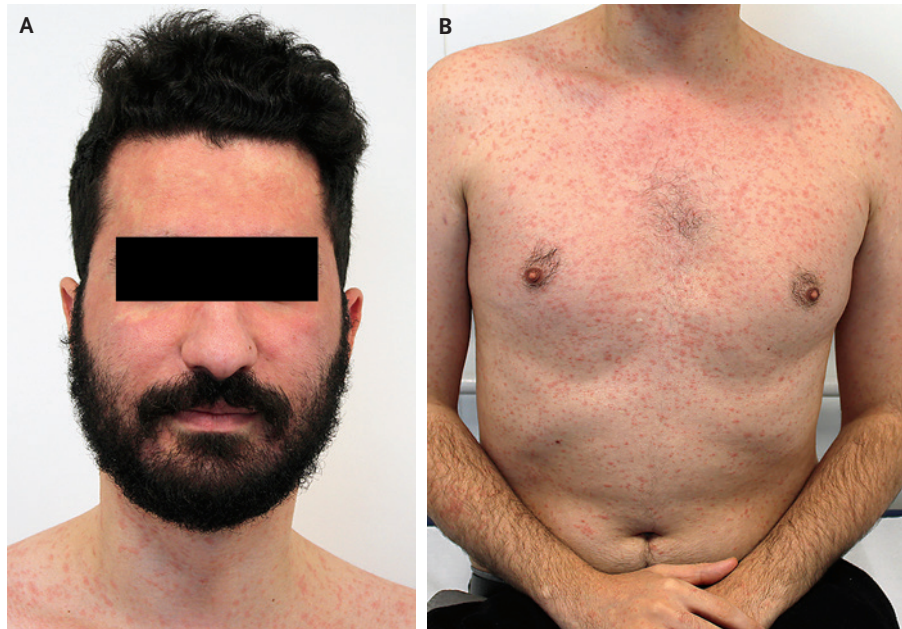


IMAGES IN CLINICAL MEDICINE

Primary Measles Infection



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A PREVIOUSLY HEALTHY 26-YEAR-OLD MAN PRESENTED TO THE DERMATOLOGY emergency clinic with a 5-day history of fever, nausea, and vomiting and a 2-day history of an itchy rash. The rash had started behind his ears and then spread to his face and entire body within 24 hours. The patient was a refugee from a Middle Eastern country and had not been vaccinated against measles, mumps, or rubella. The physical examination was notable for an exanthem consisting of innumerable minute macules and subtly elevated papules on the face (Panel A) and trunk (Panel B). Well-demarcated erosions were also present on the buccal mucosa on both sides of the mouth; the appearance of the erosions was consistent with Koplik spots. Serologic testing for antimeasles IgM and IgG was positive. A quantitative real-time polymerase-chain-reaction assay for measles virus performed on a blood sample was also positive. A diagnosis of primary measles infection was made. Treatment with antipyretic medications was initiated. Public health authorities were notified, and contact tracing was initiated. Five additional cases of measles were identified, but no larger outbreak occurred. Two weeks after presentation, the patient had recovered without complications.

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